

LRI Emergency Department Shoulder dislocation (glenohumeral joint)

To be used at the point of
suspected diagnosis

For patients in the adult ED
and 16 and 17 year olds in the
CED when appropriate clinical
expertise is available

Concomitant injuries or acute
conditions to be managed
separately

Disclaimer: This is a clinical template;
clinicians should always use judgment
when managing individual patients

Reapproved by ED guidelines committee on 06May26
Review due May 2031 . Trust Ref: C75/2016

Patient details

Full
name

DoB

S number

(use addressograph if available)

① Analgesia bundle

Offer pentrox ([prescribing aid](#)
available from the ED print kiosk
menu 'Injuries > ED Proformas')
or nitrous oxide as needed until
pain well controlled through:

Systemic analgesia bundle

- Paracetamol
- Ibuprofen if safe (see box 2)
- Oramorph

Consider intra-articular block
(**IAB**, see box 3) if skin intact
and obvious anterior dislocation
If pain score still > 6/10 and
patient not suitable for **IAB**,
consider adding morphine IV

② NSAID cautions

DO NOT prescribe NSAID if

- ☐ Patient allergic to any NSAID
- ☐ Exacerbation of asthma after
previous use of any NSAID
- ☐ Known current peptic ulcer
- ☐ History or current clinical signs
of CAD or heart failure
- ☐ Current antiplatelet or
anticoagulant use
- ☐ Chronic kidney disease (CKD)
- ☐ Acute illness likely to cause AKI

③ Intra-articular block

Competent practitioners only

Record verbal consent; mention
possible adverse outcomes below

- Block failure (i.e. need to add
procedural sedation); 1 in 20
- Joint infection; < 1 in 4000

Use plain lidocaine **1%** solution

Weight	Dose	Volume
40 - 49.9kg	120mg	12 mL
50 - 59.9kg	150mg	15 mL
60 - 66.6kg	180mg	18 mL
over 66.6kg	200mg	20 mL (Max)

⌚ **Wait 15-20min** for peak effect

④ Aftercare bundle

- ☐ Immobilise shoulder in a polysling
- ☐ Reassess and document absence
of new neurovascular deficit
(**NB:** Inform EPIC if new deficit!)
- ☐ Obtain shoulder radiographs to
demonstrate successful reduction
- ☐ Manage any associated injuries
and/or illness as appropriate
- ☐ Assess if patient is able to carry out
their ADLs and consider referral to
EDU/FES/FSDEC/AFU if indicated
Provide patient with the PILs below
- ☐ [Shoulder dislocation](#)
- ☐ [Fracture clinic](#)
- ☐ Refer to fracture clinic via [eTrauma](#);
state if dislocation first or recurrent
- ☐ Unless admitted, suggest suitable
over-the-counter analgesics or
prescribe TTO as indicated
- ☐ Offer 2-week fit note for manual
work or jobs involving lifting/
overhead tasks (office/desk
workers may self-certify)

- Clinician to assess within 15min of arrival
- Document arm neurovascular status
- Check for associated injuries / illness
- Analgesia within 30min of arrival (see boxes 1-3)

Definite recurrent
anterior dislocation?

N

Injury due to trauma?

Y

Request plain shoulder radiographs (should be taken
within 30min of request and within 60min of arrival)

Dislocation confirmed?

N

Manage as
appropriate

Significant fracture?

Y

i.e. anything other
than tubercle
avulsions or small
glenoid fractures

Simple anterior dislocation
and skin intact?

N

Undertake reduction within 90min of arrival using
pentrox or nitrous oxide (and, if analgesia not
sufficient, consider IAB as per box 3 if not already)

Reduction successful?

N

Consider e-referral
to orthopaedics team
for further attempt
without procedural
sedation if high ER
occupancy is
anticipated to cause
significant delay

Attempt reduction under
procedural sedation **in ER** (if first
attempt: within 90min of arrival);
use [ED procedural sedation chart](#)

Reduction successful?

N

MUA
needed

Aftercare bundle
(see box 4)

eRefer to on-call
orthopaedic team

Patient was
managed
by

Print name

Signature

Position

Date